

Zylo Health

AI assisted radiology triage for tier 2 and tier 3 hospitals

Seed round, July 2026

Kochi, Kerala

The problem

- A 180 bed hospital in Kottakkal runs 6,000 scans a month and has one visiting radiologist.
- Studies are read in the order they arrive, not in the order they matter.
- A bleed found on Thursday was often taken on Tuesday.
- India has roughly one radiologist for every 100,000 people. Tier 2 has far fewer.

What we do

- Zylo reads every study as it lands and ranks the worklist by urgency.
- Critical findings are pushed to the radiologist's phone with the key slice attached.
- The radiologist still reads and still signs. We change the order, not the diagnosis.
- Median time to flag a critical study is 48 seconds.

Why this is the right wedge

- Triage does not require us to be right about everything. It requires us to be right about urgency.
- It fits the regulatory path: a Class B software device, not a diagnostic claim.
- It is bought by the hospital administrator, whose problem is coverage, not accuracy.
- It makes one radiologist safely cover more hospitals, which is the actual constraint.

Traction

- 46,200 scans processed in July 2026.
- 11 hospitals live across Kerala and Tamil Nadu, plus a four hospital district pilot.
- Contracted ARR of Rs 1.87 crore.
- 889 critical findings flagged in July alone.

Clinical evidence

- Internal validation on 24,000 held out studies: sensitivity 94.1 percent, specificity 88.7 percent.
- Prospective arm at two hospitals since March 2026, 11,400 studies.
- Mean reduction in time to radiologist review for critical studies: 3 hours 41 minutes.
- Two peer reviewed abstracts submitted, one accepted.

Regulatory

- CDSCO Class B application filed 14 April 2026, acknowledgement MD 15 2026 0417.
- Quality management system audited to ISO 13485 in February 2026.
- Clinical evaluation report and risk file complete.
- Expected decision window is 6 to 9 months from filing.

Business model

- Annual hospital contract, priced per scan on committed volume.
- Realised price Rs 34 per scan. Gross contribution Rs 25.80.
- No hardware. Deployment is a PACS integration, median 9 days.
- Net revenue retention 131 percent, driven by scan volume growth inside existing hospitals.

Use of funds

- Rs 8.0 crore, expected to last 20 months post round.
- Rs 3.0 crore: regulatory completion and the prospective multi site study.
- Rs 2.4 crore: clinical sales team for Tamil Nadu and Karnataka.
- Rs 1.8 crore: engineering, principally scanner vendor calibration.
- Rs 0.8 crore: reserve.

The team

- Dr Ann Mary Varghese, CEO. Consultant radiologist, over 90,000 studies read.
- Sreejith Nair, CTO. Six years building imaging models, two cleared CDSCO.
- Dr Vivek Menon, Clinical Affairs. Two prospective device studies to submission.
- 14 full time, including three radiologists on retainer.